

Ephedrine: Drug information - UpToDate

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For additional information see "Ephearine: Patient drug information" and "Ephearine: Pediatric drug information"

For abbreviations, symbols, and age group definitions show table

Brand Names: US

- Akovaz;
- Bronkaid Max [OTC];
- Emerphea;
- Primatene [OTC];
- Rezipres [DSC]

Pharmacologic Category

- Alpha-/Beta- Agonist

Dosing: Adult

Asthma, mild intermittent symptoms

Asthma, mild intermittent symptoms: Note: Clinical practice guidelines do not recommend nonselective beta agonists or oral beta-2 agonists, including oral ephearine, for routine management and treatment of asthma due to their potential for excessive cardiac stimulation, especially in high doses (Ref).

Oral: OTC labeling:

Bronkaid Max: 25 mg every 4 hours as needed; maximum 150 mg per 24 hours.

Primatene: 12.5 to 25 mg every 4 hours as needed; maximum 150 mg per 24 hours.

Hypotension, anesthesia induced

Hypotension, anesthesia induced: Note: Ephearine sulfate 10 mg is equivalent to ephearine base 7.6 mg; ephearine hydrochloride 9.4 mg is equivalent to ephearine base 7.7 mg.

Akovaz, Emerphea (ephearine sulfate): IV: Initial: 5 to 10 mg; repeat as needed to maintain BP (maximum total cumulative dose: 50 mg).

Rezipres (ephedrine hydrochloride): IV: Initial: 4.7 to 9.4 mg; repeat as needed to maintain BP (maximum total cumulative dose: 47 mg).

Postoperative nausea and vomiting

Postoperative nausea and vomiting (prevention) (off-label use): IM: 0.5 mg/kg at the end of surgery (Ref).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling.

Dosing: Liver Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling.

Dosing: Older Adult

Initiate at lower end of dosing range. Refer to adult dosing.

Dosing: Pediatric

(For additional information see "Ephedrine: Pediatric drug information")

Hypotension, anesthesia induced

Hypotension, anesthesia induced: Limited data available:

Note: For the parenteral product, two salts are available (sulfate and hydrochloride); ephedrine sulfate 10 mg is equivalent to ephedrine base 7.6 mg; ephedrine hydrochloride 9.4 mg is equivalent to ephedrine base 7.7 mg; clinical relevance of difference between salt forms is unknown. Parenteral products may be available in significantly different concentrations (eg, 50 mg/mL and 5 mg/mL); use caution.

Infants, Children, and Adolescents: Slow IV push: 0.1 to 0.3 mg/kg/dose; use the lowest effective dose; usual adult dose range: 5 to 10 mg/dose; repeat as needed to maintain blood pressure; maximum total dose: 50 mg (Ref).

Asthma, intermittent; acute symptom relief

Asthma, intermittent; acute symptom relief (OTC labeling): **Note:** Oral bronchodilator therapy, such as ephedrine, is not recommended for treatment of asthma due to slow onset of action and high rate of adverse effects (Ref).

Children ≥12 years and Adolescents: Oral: 12.5 to 25 mg every 4 hours as needed; maximum daily dose: 150 mg per 24 hours (Ref).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Pediatric

There are no dosage adjustments provided in the manufacturer's labeling; use with caution. Elimination of ephedrine may be delayed in patients with kidney impairment; the manufacturer recommends monitoring carefully for adverse events.

Dosing: Liver Impairment: Pediatric

There are no dosage adjustments provided in the manufacturer's labeling.

Adverse Reactions

The following adverse drug reactions are derived from product labeling unless otherwise specified.

Postmarketing:

Cardiovascular: Bradycardia, hypertension (reactive), irregular pulse (RR interval variability), palpitations, tachycardia, ventricular ectopy

Gastrointestinal: Nausea, vomiting

Nervous system: Dizziness, restlessness

Miscellaneous: Tachyphylaxis

Contraindications

IV: There are no contraindications listed in the manufacturer's labeling.

Oral: OTC labeling: When used for self-medication, do not use if history of allergic reaction to epinephrine or any component of the formulation, concurrently with or within 2 weeks of discontinuing a monoamine oxidase inhibitor, or if you do not have asthma.

Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Warnings/Precautions

Concerns related to adverse effects:

- Cardiovascular effects: May cause hypertension if used prophylactically for hypotension (only indicated for treatment of hypotension).

Disease-related concerns:

- Renal impairment: Use with caution in patients with renal impairment; increased elimination half-life may occur. Carefully monitor patients with renal impairment for adverse reactions.

Special populations:

- Older adult: Use with caution in the elderly.

Other warnings/precautions:

- Self-medication (OTC use): Prior to self-medication, patients should contact health care provider if they have diabetes, heart disease, hypertension, narrow angle glaucoma, psychiatric or emotional conditions, seizures, thyroid disease, trouble urinating due to an enlarged prostate, ever been hospitalized for asthma, are taking any medications for asthma, depression, obesity, psychiatric or emotional conditions, or are taking any medications that contain caffeine, epinephrine, phenylephrine, or pseudoephedrine (eg, allergy, cough/cold, or pain medications). Avoid foods or beverages that contain caffeine and dietary supplements that have a stimulant effect. Discontinue use and contact health care provider if asthma is not better in 60 minutes or gets worse, >2 asthma attacks in a week, need >150 mg in 24 hours or >100 mg in 24 hours for ≥3 days a week, or if insomnia, nervousness, a rapid heartbeat, seizures, or tremors occur.
- Tolerance: Tachyphylaxis and tolerance may develop with repeated, prolonged, or excessive administration; temporary cessation of therapy restores its effectiveness.

Dosage Forms: US

Excipient information presented when available (limited, particularly for generics); consult specific product labeling. [DSC] = Discontinued product

Solution, Intravenous, as hydrochloride [preservative free]:

Rezipres: 23.5 mg/5 mL (5 mL [DSC])

Rezipres: 47 mg/10 mL (10 mL [DSC]) [sulfite free]

Solution, Injection, as sulfate [preservative free]:

Generic: 50 mg/mL (1 mL [DSC])

Solution, Intravenous, as sulfate:

Akovaz: 50 mg/mL (1 mL)

Generic: 50 mg/mL (1 mL)

Solution, Intravenous, as sulfate [preservative free]:

Emerphea: 5 mg/mL (10 mL)

Generic: 5 mg/mL (10 mL); 50 mg/mL (1 mL)

Solution Prefilled Syringe, Intravenous [preservative free]:

Emerphea: 50 mg/10 mL (10 mL) [latex free]

Generic: 50 mg/10 mL (10 mL)

Solution Prefilled Syringe, Intravenous, as sulfate [preservative free]:

Akovaz: 25 mg/5 mL (5 mL)

Emerphed: 25 mg/5 mL (5 mL) [latex free]

Generic: 25 mg/5 mL (5 mL)

Tablet, Oral, as hydrochloride:

Primatene: 12.5 mg [contains fd&c yellow #6(sunset yellow)alumin lake, quinoline (d&c yellow #10) aluminum lake]

Tablet, Oral, as sulfate:

Bronkaid Max: 25 mg [dye free]

Generic Equivalent Available: US

May be product dependent

Pricing: US

Solution (Akovaz Intravenous)

50 mg/mL (per mL): \$18.48

Solution (Emerphea Intravenous)

5 mg/mL (per mL): \$3.48

Solution (ePHEDrine Sulfate (Pressors) Intravenous)

5 mg/mL (per mL): \$3.48

50 mg/mL (per mL): \$6.28 - \$59.11

Solution Prefilled Syringe (Akovaz Intravenous)

25 mg/5 mL (per mL): \$2.96

Solution Prefilled Syringe (Emerphea Intravenous)

25 mg/5 mL (per mL): \$9.00

50 mg/10 mL (per mL): \$6.24

Solution Prefilled Syringe (ePHEDrine Sulfate (Pressors) Intravenous)

25 mg/5 mL (per mL): \$2.96

Tablets (Bronkaid Max Oral)

25 mg (per each): \$0.19

Disclaimer: A representative AWP (Average Wholesale Price) price or price range is provided as reference price only. A range is provided when more than one manufacturer's AWP price is available and uses the low and high price reported by the manufacturers to determine the range. The pricing data should be used for benchmarking purposes only, and as such should not be used alone to set or adjudicate any prices for reimbursement or purchasing functions or considered to be an exact price for a single product and/or manufacturer. Medi-Span expressly disclaims all warranties of any kind or nature, whether express or implied, and assumes no liability with respect to accuracy of price or price range data published in its solutions. In no event shall Medi-Span be liable for special, indirect,

incidental, or consequential damages arising from use of price or price range data. Pricing data is updated monthly.

Dosage Forms: Canada

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Solution, Injection, as sulfate:

Generic: 50 mg/mL (1 mL)

Administration: Adult

IV: Administer as an IV bolus. Verify formulation prior to administration; available in vials requiring further dilution and also premixed syringes and vials (requiring no further dilution).

IM: For postoperative nausea and vomiting (off-label use), administer IM (Ref).

Preparation for Administration: Adult

IV: Using a 50 mg/mL vial, withdraw 50 mg and dilute with 9 mL of D5W or NS to final concentration of 5 mg/mL. Premixed syringes and vials requiring no further dilution are also available.

IM: For postoperative nausea and vomiting (off-label use), dilute with NS (Ref).

Administration: Pediatric

Parenteral: IV: Administer by slow IV push. Verify formulation prior to administration; available in vials requiring further dilution and also premixed syringes and vials (requiring no further dilution).

Preparation for Administration: Pediatric

Parenteral: IV:

Ephedrine sulfate: Dilute 50 mg (1 mL) in 9 mL of D5W or NS; final concentration: 5 mg/mL.

Ephearine hydrochloride:

47 mg/mL solution: Must be diluted prior to administration; dilute 47 mg (1 mL) in 9 mL of D5W or NS; final concentration: 4.7 mg/mL.

9.4 mg/mL solution: May be administered undiluted or diluted by adding 47 mg (5 mL) to 5 mL of D5W or NS; final concentration: 4.7 mg/mL.

Note: In one pediatric hypotension trial, ephearine hydrochloride doses were diluted to 1 mg/mL and 2 mg/mL; however, initial product concentration and diluent were not reported (Ref).

Storage/Stability

IV:

Akovaz, Emerphed (ephedrine sulfate): Store at 25°C (77°F); excursions are permitted between 15°C and 30°C (59°F and 86°F). Single dose only; discard any unused portion. Store Emerphed in original container prior to use.

Rezipres (ephedrine hydrochloride): Store at 20°C to 25°C (68°F to 77°F); excursions are permitted between 15°C and 30°C (59°F and 86°F). Single dose only; discard any unused portion.

Oral: Store at 20°C to 25°C (68°F to 77°F).

Use: Labeled Indications

OTC: Asthma, mild intermittent symptoms: Oral: Temporary relief of mild symptoms (shortness of breath, chest tightness, wheezing) associated with intermittent asthma.

Note: Clinical practice guidelines do not recommend nonselective beta agonists or oral beta-2 agonists, including oral ephearine, for routine management and treatment of asthma due to their potential for excessive cardiac stimulation, especially in high doses (GINA 2024; NAEPP 2007).

Rx: Hypotension, anesthesia-induced: IV: Treatment of anesthesia-induced hypotension.

Use: Off-Label: Adult

Postoperative nausea and vomiting (prevention)

Medication Safety Issues

Sound-alike/look-alike issues:

Akovaz may be confused with Adrenalin

EPHEDrine may be confused with Epifrin, EPINEPHrine

Rezipres (ePHEDrine hydrochloride) may be confused with ePHEDrine sulfate products

High alert medication:

The Institute for Safe Medication Practices (ISMP) includes this medication among its list of drug classes (adrenergic agonist, IV) which have a heightened risk of causing significant patient harm when used in error (ISMP List of High-Alert Medications in Acute Care Settings ©ISMP 2024).

Metabolism/Transport Effects

None known.

Drug Interactions

(For additional information: Launch drug interactions program)

Note: Interacting drugs may **not be individually listed below** if they are part of a group interaction (eg, individual drugs within “CYP3A4 Inducers [Strong]” are NOT listed). For a complete list of drug interactions by individual drug name and detailed management recommendations, use the drug interactions program by clicking on the “Launch drug interactions program” link above.

Alpha1-Blockers: May decrease therapeutic effects of Alpha-/Beta-Agonists. *Risk C: Monitor*

Atomoxetine: May increase hypertensive effects of Sympathomimetics. Atomoxetine may increase tachycardic effects of Sympathomimetics. *Risk C: Monitor*

Atropine (Systemic): May increase therapeutic effects of EPHEDrine (Systemic). *Risk C: Monitor*

Benzylpenicilloyl Polylysine: Coadministration of Alpha-/Beta-Agonists and Benzylpenicilloyl Polylysine may alter diagnostic results. Management: Consider delaying skin testing until alpha/beta agonists are no longer required and their effects have diminished, or use a histamine skin test as a positive control to assess a patient’s ability to mount a wheal and flare response. *Risk D: Consider Therapy Modification*

Beta-Blockers: May decrease therapeutic effects of EPHEDrine (Systemic). *Risk C: Monitor*

Bitter Orange: May increase adverse/toxic effects of Sympathomimetics. *Risk C: Monitor*

Bornaprine: Sympathomimetics may increase anticholinergic effects of Bornaprine. *Risk C: Monitor*

Bromocriptine: May increase hypertensive effects of Alpha-/Beta-Agonists. Management: Consider alternatives to this combination when possible. If combined, monitor for hypertension and tachycardia, and do not coadminister these agents for more than 10 days. *Risk D: Consider Therapy Modification*

Cannabinoid-Containing Products: May increase tachycardic effects of Sympathomimetics. *Risk C: Monitor*

Cardiac Glycosides: EPHEDrine (Systemic) may increase arrhythmogenic effects of Cardiac Glycosides. *Risk C: Monitor*

Chloroprocaine (Systemic): May increase hypertensive effects of Alpha-/Beta-Agonists. *Risk C: Monitor*

CloNIDine: May increase therapeutic effects of EPHEDrine (Systemic). *Risk C: Monitor*

CloZAPine: May decrease therapeutic effects of Alpha-/Beta-Agonists. *Risk C: Monitor*

Cocaine (Topical): May increase hypertensive effects of Sympathomimetics. Management: Consider alternatives to use of this combination when possible. Monitor closely for substantially increased blood pressure or heart rate and for any evidence of myocardial ischemia with concurrent use. *Risk D: Consider Therapy Modification*

DexAMETHasone (Systemic): EPHEDrine (Systemic) may decrease serum concentrations of DexAMETHasone (Systemic). *Risk C: Monitor*

Dihydralazine: Sympathomimetics may decrease therapeutic effects of Dihydralazine. *Risk C: Monitor*

Doxapram: Sympathomimetics may increase adverse/toxic effects of Doxapram. *Risk C: Monitor*

Doxofylline: Sympathomimetics may increase adverse/toxic effects of Doxofylline. *Risk C: Monitor*

Droxidopa: Sympathomimetics may increase hypertensive effects of Droxidopa. *Risk C: Monitor*

Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates): May increase vasoconstricting effects of Alpha-/Beta-Agonists. *Risk X: Avoid*

Esketamine (Injection): May increase adverse/toxic effects of Sympathomimetics. Specifically, the risk for elevated heart rate, hypertension, and arrhythmias may be increased. *Risk C: Monitor*

FentaNYL: Decongestants may decrease serum concentrations of FentaNYL. *Risk C: Monitor*

Guanethidine: May increase hypertensive effects of Sympathomimetics. Guanethidine may increase arrhythmogenic effects of Sympathomimetics. *Risk C: Monitor*

Hexoprenaline: May increase adverse/toxic effects of Alpha-/Beta-Agonists. *Risk X: Avoid*

Hyaluronidase: May increase vasoconstricting effects of Alpha-/Beta-Agonists. Management: Do not use hyaluronidase to enhance the dispersion or absorption of alpha-/beta-agonists. Use of hyaluronidase for other purposes in patients receiving alpha-/beta-agonists may be considered as clinically indicated. *Risk D: Consider Therapy Modification*

Iobenguane Radiopharmaceutical Products: Alpha-/Beta-Agonists (Indirect-Acting) may decrease therapeutic effects of Iobenguane Radiopharmaceutical Products. Management: Discontinue all drugs that may inhibit or interfere with catecholamine transport or uptake for at least 5 biological half-lives before iobenguane administration. Do not administer these drugs until at least 7 days after each iobenguane dose. *Risk X: Avoid*

Kratom: May increase adverse/toxic effects of Sympathomimetics. *Risk X: Avoid*

Linezolid: May increase hypertensive effects of Sympathomimetics. Management: Consider initial dose reductions of sympathomimetic agents, and closely monitor for enhanced blood pressure elevations, in patients receiving linezolid. *Risk D: Consider Therapy Modification*

Lisuride: May increase hypertensive effects of Alpha-/Beta-Agonists. Management: Consider alternatives when possible, as product labeling states the concomitant use of lisuride with alpha-/beta-agonists is not recommended. If combined, monitor for increases in blood pressure or hypertensive crises. *Risk D: Consider Therapy Modification*

Metergoline: May increase adverse/toxic effects of Alpha-/Beta-Agonists (Indirect-Acting). *Risk C: Monitor*

Monoamine Oxidase Inhibitors: May increase hypertensive effects of Alpha-/Beta-Agonists (Indirect-Acting). While linezolid is expected to interact via this mechanism, management recommendations differ from other monoamine oxidase inhibitors. Refer to linezolid specific monographs for details. *Risk X: Avoid*

Oxytocin: May increase hypertensive effects of EPHEDrine (Systemic). *Risk C: Monitor*

Pergolide: May increase hypertensive effects of Alpha-/Beta-Agonists. *Risk C: Monitor*

Propofol: May increase therapeutic effects of EPHEDrine (Systemic). *Risk C: Monitor*

QuiNIDine: May decrease therapeutic effects of EPHEDrine (Systemic). EPHEDrine (Systemic) may decrease therapeutic effects of QuiNIDine. *Risk C: Monitor*

Reserpine: May decrease therapeutic effects of Alpha-/Beta-Agonists (Indirect-Acting). *Risk C: Monitor*

Serotonin/Norepinephrine Reuptake Inhibitor: May increase tachycardic effects of Alpha-/Beta-Agonists. Serotonin/Norepinephrine Reuptake Inhibitor may increase vasopressor effects of Alpha-/Beta-Agonists. Management: If possible, avoid coadministration of direct-acting alpha-/beta-agonists and serotonin/norepinephrine reuptake inhibitors. If coadministered, monitor for increased sympathomimetic effects (eg, increased blood pressure, chest pain, headache). *Risk D: Consider Therapy Modification*

Solriamfetol: Sympathomimetics may increase hypertensive effects of Solriamfetol. Sympathomimetics may increase tachycardic effects of Solriamfetol. *Risk C: Monitor*

Sympathomimetics: May increase adverse/toxic effects of Sympathomimetics. *Risk C: Monitor*

Tedizolid: May increase adverse/toxic effects of Sympathomimetics. Specifically, the risk for increased blood pressure and heart rate may be increased. *Risk C: Monitor*

Theophylline Derivatives: EPHEDrine (Systemic) may increase adverse/toxic effects of Theophylline Derivatives. *Risk C: Monitor*

Theophylline: May increase stimulatory effects of EPHEDrine (Systemic). *Risk C: Monitor*

Thyroid Products: May increase adverse/toxic effects of Sympathomimetics. Specifically, the risk of hypertension, tachycardia, and coronary insufficiency may be increased in patients with coronary artery disease. *Risk C: Monitor*

Tranylcypromine: May increase hypertensive effects of Alpha-/Beta-Agonists (Indirect-Acting). *Risk X: Avoid*

Tricyclic Antidepressants: May increase vasopressor effects of Alpha-/Beta-Agonists. Management: Avoid, if possible, the use of alpha-/beta-agonists in patients receiving tricyclic antidepressants. If combined, monitor for evidence of increased pressor effects and consider reductions in initial dosages of the alpha-/beta-agonist. *Risk D: Consider Therapy Modification*

Vasopressin: Alpha-/Beta-Agonists (Direct-Acting) may increase hypertensive effects of Vasopressin. The effect of other hemodynamic parameters may also be enhanced. *Risk C: Monitor*

Pregnancy Considerations

Metabolic acidosis has been reported in neonates following maternal use of epinephrine; monitor.

Untreated maternal hypotension during cesarean delivery is associated with adverse events, including maternal nausea and vomiting, and bradycardia and acidosis in the fetus. Epinephrine injection is used at delivery for the prevention and/or treatment of maternal hypotension associated with spinal anesthesia in patients undergoing cesarean delivery (ACOG 2019). Serious postpartum hypertension and possibly stroke may occur if administered with oxytocic medications.

Breastfeeding Considerations

Epinephrine is present in breast milk.

According to the manufacturer, the decision to breastfeed during therapy should consider the risk of infant exposure, the benefits of breastfeeding to the infant, and the benefits of treatment to the mother.

Monitoring Parameters

BP, pulse; monitor patients with renal impairment for adverse reactions.

Mechanism of Action

Releases tissue stores of norepinephrine and thereby produces an alpha- and beta-adrenergic stimulation; longer-acting and less potent than epinephrine

Pharmacokinetics (Adult Data Unless Noted)

Onset: IM: Within 10 to 20 minutes.

Metabolism: Minimally hepatic; metabolites include p-hydroxyephedrine, p-hydroxynorephedrine, norephedrine.

Half-life elimination: Dependent upon urinary pH; Urine pH 5: ~3 hours; Urine pH 6.3: ~6 hours.

Excretion: Urine (primarily unchanged; dependent upon urinary pH with greatest excretion in acid pH).

Pharmacokinetics: Additional Considerations (Adult Data Unless Noted)

Altered kidney function: Elimination half-life may be increased.

Patient Counseling Points

(For additional information see "Epinephrine: Patient drug information")

What is this drug used for?

Tablets:

- It is used to treat asthma.

Injection:

- It is used to treat low blood pressure.
- It may be given to you for other reasons. Talk with the doctor.

All drugs may cause side effects. However, many people have no side effects or only have minor side effects. Call your doctor or get medical help if any of these side effects or any other side effects bother you or do not go away:

- Dizziness or headache
- Sweating a lot
- Restlessness
- Trouble sleeping
- Feeling nervous and excitable
- Upset stomach or throwing up
- Not hungry

WARNING/CAUTION: Even though it may be rare, some people may have very bad and sometimes deadly side effects when taking a drug. Tell your doctor or get medical help right away if you have any of the following signs or symptoms that may be related to a very bad side effect:

- High blood pressure like very bad headache or dizziness, passing out, or change in eyesight
- Weakness on 1 side of the body, trouble speaking or thinking, change in balance, drooping on one side of the face, or blurred eyesight
- Chest pain or pressure
- Fast, slow, or abnormal heartbeat
- Trouble passing urine
- Shortness of breath
- Seizures

- Signs of an allergic reaction, like rash; hives; itching; red, swollen, blistered, or peeling skin with or without fever; wheezing; tightness in the chest or throat; trouble breathing, swallowing, or talking; unusual hoarseness; or swelling of the mouth, face, lips, tongue, or throat.

Note: This is not a comprehensive list of all side effects. Talk to your doctor if you have questions.

Consumer Information Use and Disclaimer: This information should not be used to decide whether or not to take this medicine or any other medicine. Only the healthcare provider has the knowledge and training to decide which medicines are right for a specific patient. This information does not endorse any medicine as safe, effective, or approved for treating any patient or health condition. This is only a limited summary of general information about the medicine's uses from the patient education leaflet and is not intended to be comprehensive. This limited summary does NOT include all information available about the possible uses, directions, warnings, precautions, interactions, adverse effects, or risks that may apply to this medicine. This information is not intended to provide medical advice, diagnosis or treatment and does not replace information you receive from the healthcare provider. For a more detailed summary of information about the risks and benefits of using this medicine, please speak with your healthcare provider and review the entire patient education leaflet.

Brand Names: International

International Brand Names by Country

For country code abbreviations (show table)

- (AT) Austria: Ephedrin meduna | Ephearine medicamentum | Ephedrinhydrochlorid Aguettant;
- (AU) Australia: Ephedrine aguettant | Ephedrine hydrochloride myx | Ephearine hydrochloride sxp;
- (BD) Bangladesh: Brodrine | Ephedrin;
- (BE) Belgium: Ephearine HCl Aguettant;
- (BG) Bulgaria: Ephedrin;
- (CH) Switzerland: Ephedrin labatec | Ephearine aguettant;
- (CL) Chile: Efedrosan;
- (CN) China: Ephedrine;
- (CO) Colombia: Efedrina sulfato;
- (CZ) Czech Republic: Ephedrin;
- (DE) Germany: Ephedrin Carinopharm | Ephedrin meduna | Ephedrinhydrochlorid Aguettant;
- (EE) Estonia: Ephedrine sintetica | Ephedrini hcl;
- (EG) Egypt: Ephedrine;
- (ES) Spain: Efedrina level | Hidrocloruro de Efedrina | Hidrocloruro de efedrina genfarma;
- (FI) Finland: Efedrin | Efedrin abboxia | Efedrin aguettant | Efedrin stragen | Efedrin unimedic | Ephedrine sintetica;
- (FR) France: Ephedrine Arrow | Ephedroides;
- (GB) United Kingdom: Ephedrine hydrate;
- (HK) Hong Kong: Ephedrine;
- (HU) Hungary: Ephedrin pharmexim | Epherit;
- (ID) Indonesia: Asthma | Efedrin | Ephedrin Hcl | Ephearine | Ephedrine HCL | Ephedrine(b) | Erladrine;
- (IT) Italy: Efedri C | Efedrina aguettant;
- (JP) Japan: Ephearine | Ephedrine hcl taisho | Ephearine nagai;
- (KR) Korea, Republic of: Ephearine;
- (KW) Kuwait: Ephearine HCL;
- (LT) Lithuania: Ephedrin;
- (LV) Latvia: Ephedrin | Ephearine sintetica;
- (MY) Malaysia: Ephearine;
- (NL) Netherlands: Efedrine added pharma | Efedrine hcl | Efedrine hcl aguettant;
- (NO) Norway: Efedrin | Efedrin abboxia | Efedrin aguettant | Efedrin NAF | Efedrin stragen |

- Efedrin unimedic | Ephearine sintetica;
- (NZ) New Zealand: Ephearine Hydrochloride Aguettant;
- (PA) Panama: Efedrina aguettant;
- (PH) Philippines: Hizon ephedrine sulfate;
- (PK) Pakistan: Efed | Ephearine;
- (PL) Poland: Ephedrinum hydrochloricum;
- (PR) Puerto Rico: Akovaz | Emerphea | Primatene asthma | Rezipres;
- (PT) Portugal: Efedrina aguettant | Efedrina level;
- (RO) Romania: Efedrina Arena;
- (RU) Russian Federation: Ephedrin;
- (SE) Sweden: Efedrin abboxia | Efedrin stragen | Efedrin unimedic | Ephearine sintetica;
- (SG) Singapore: Ephedrine;
- (SI) Slovenia: Efedrin sintetica | Ephearine HCl Sterop;
- (SK) Slovakia: Ephedrin;
- (TH) Thailand: Ephedrine | Ephredrine;
- (TR) Turkey: Efedrin | Ephedrin;
- (TW) Taiwan: E Fdrine | Ephedrine | Ephedrinum;
- (UA) Ukraine: Ephedrin;
- (UG) Uganda: Ephearine | Ephearine aguettant;
- (VE) Venezuela, Bolivarian Republic of: Efedrina

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